Outlook

Qlaim1816 - Mr & Mrs Scarr - Re: Contamination Risk and Material Disposal: Medical and Environmental Grounds Relating to Mrs Louisa Scarr

From Andy Lyne <Andy.Lyne@qlaims.com>
Date Fri 04/07/2025 15:09
To Elizabeth Turner <elizabeth.turner@davies-group.com>; Adjuster.Correspondence <adjuster.correspondence@davies-group.com>
Cc Louisa Scarr <scarrlouisa@gmail.com>; Claims <Claims@qlaims.com>; Nicholas Scarr <nscarr@quantlogic.com>

Afternoon Lizzie,

I hope you're well.

Further to our recent discussion regarding health concerns by our clients please see an email below which we have received by Mr Scarr.

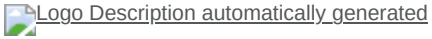
Can I ask you to discuss the content the email with insurers.

Best Regards

Andy Lyne
Qlaims Adjuster

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From: Nicholas Scarr <nscarr@quantlogic.com>
Sent: 01 July 2025 18:33
To: Andy Lyne <Andy.Lyne@qlaims.com>; Claims <Claims@qlaims.com>
Cc: Louisa Scarr <scarrlouisa@gmail.com>; Claims <Claims@qlaims.com>; Sarah Romain <sarah.romain@qlaims.com>
Subject: Contamination Risk and Material Disposal: Medical and Environmental Grounds Relating to Mrs Louisa Scarr

Dear Andy,
This correspondence sets out our unequivocal position in respect of the health and environmental risks posed by the sewage contamination incident at our property, and the consequent requirement for the removal of specific contaminated materials.

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1. Medical Vulnerability of Mrs Louisa Scarr

Mrs Louisa Scarr's clinical history places her among the highest-risk individuals in relation to environmental exposure to microbial and opportunistic pathogens. Her medical record includes:

- **Clostridioides difficile (C. difficile)** – a spore-forming, toxin-producing bacterium capable of surviving in environmental reservoirs and highly resistant to standard disinfectants;
- **Klebsiella pneumoniae (NDM strain)** – a carbapenem-resistant "superbug" classified as critical priority by the World Health Organization due to limited treatment options and high mortality;
- **Sepsis** – which required critical care intervention and was life-threatening to the extent that last rites were administered in 2013.

These infections not only compromised Mrs Scarr's gastrointestinal and systemic immune defences, but have left her clinically and immunologically vulnerable to further microbial exposure. The persistent risk of recurrence, particularly from exposure to spores and drug-resistant pathogens, is material and supported by both NHS infection control guidance and global clinical literature.
Importantly, no decontamination method can offer a 100% guarantee of eliminating all microbial risks, especially where porous and absorbent materials are involved, or where contamination has persisted for extended periods. This places Mrs Scarr at demonstrable and unacceptable risk, should exposure to residual contamination occur.

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2. Timeline and Environmental Conditions

- **Initial Exposure Period**: The ground floor was exposed to raw sewage for approximately fifteen weeks. Apart from a superficial surface clean in the early stages, the contamination remained untreated and pooled beneath floorboards until Bio Response commenced professional drying and sanitation.
- **Rat Infestation**: The prolonged presence of sewage and organic waste material led to a significant rat infestation affecting both our home and neighbouring properties. This is strongly corroborated by visual evidence and pest control reports.
- **Bio Response Findings**: The specialist contractor advised unequivocally that the kitchen units had been compromised and required full removal. They further observed widespread microbial and moisture damage on both floors.
- **Strip-Out Findings**: Upon commencement of the strip-out process—more than three months post-incident—extensive dampness and persistent sewage odours were observed, with visible patches of water still present beneath surface structures.
- **Upstairs Contamination Indicators**: Moisture-related cracking on the first floor and the presence of sewage odours and dampness are indicative of airborne microbial migration and humidity-related contamination.

- **Sanitisation Limitations**: Despite the application of disinfectant solutions, an **ATP (adenosine triphosphate) reading of 27+** was recorded at the former location of the Everhot kitchen unit. This significantly exceeds acceptable hygiene thresholds and confirms the presence of biologically active contamination.
- **Drying and Scrubbing Measures**: Industrial drying machines and air scrubbers are currently deployed, yet they do not reverse the prolonged contamination exposure nor render porous materials hygienically sound.

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3. Justified Material Removal

Following both site-specific expert recommendations and recognised international remediation protocols (including guidance by the CDC, EPA, IICRC, and UK Health and Safety Executive), the following items must be permanently removed:

- **All kitchen units**, due to structural and microbial contamination in a high-humidity environment, including direct sewage proximity;
- **All soft furnishings** from both ground and first floors—this includes mattresses, upholstered furniture, carpets, and fabric-based materials. Given their absorptive nature and the likelihood of embedded spores, they cannot be rendered safe through surface disinfection.

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4. Summary Position

Our position is grounded in medical necessity and environmental fact. We reiterate that:

- No sanitisation protocol can ensure the elimination of all pathogens from previously sewage-exposed porous materials;
- Mrs Louisa Scarr's medical history necessitates a zero-tolerance approach to residual microbial risk;
- Retaining contaminated materials, even following superficial treatment, presents a material and foreseeable threat to health.

Accordingly, the removal of the items specified is both medically and environmentally essential. We appreciate your continued support and ask that this position be recorded formally in the claims file. While we are not seeking confrontation, our stance on this matter is resolute, fact-based, and not open to dilution.

Yours sincerely,

Nicholas

Nicholas Scarr